



THE RESERVE SYSTEM

BEFORE THE ALARM · CHAPTER ONE

When the Alarms Sound

*A free preview of Chapter 1 from
Before the Alarm: What the ICU Reveals About Real Longevity*

Daniel did not become sick all at once. This chapter follows the ten years before Room 7 — and introduces the idea at the center of the book: the event is dramatic, but the process is gradual. That gap is where prevention either happens, or it doesn't.

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Before the Alarm publishes September 7, 2026

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| *Longevity is not lost in events — it is lost in drift.*

Ten Years Before Room 7

Daniel used to play on Court 3.

Not competitively. Not seriously. But consistently — Tuesday evenings, the same three men, the same rotation, the same argument about whether the ball had hit in or outside the line. He kept his racquet in the back seat of his car, handle worn smooth where his palm rested. On the drive home, he would call Sarah and tell her he was running late, that he'd pick up something for dinner.

"Don't," she would say. "I already made something."

He would laugh and bring something anyway.



Daniel was forty-six the year he quit tennis.

It was not a decision, exactly. He didn't wake up one morning and choose to stop. One Tuesday evening, he was too tired from the drive home to hit the courts. The following Saturday, a work call ran long. And then, for several weeks, the racquet sat in the hallway. He kept meaning to pick it up again, but the club membership lapsed. The racquet moved to the garage. He meant to renew. He meant to find something else. He never quite got around to it.

His blood pressure, at forty-four, had read 128/84 — noted in a routine physical as "borderline." His physician documented it, advised monitoring, and recommended a follow-up in six months. Daniel did not return for fourteen months.

When he did, the number had risen. His doctor prescribed a medication. He took it sometimes, kept it where he could see it, and thought of himself as someone who took blood pressure medication. His next physical was three years later. His weight had increased, mostly around his abdomen. His resting heart rate had climbed. His sleep quality, his wife mentioned to the nurse while the physician was reviewing the chart, was getting worse. He snored badly. He was tired every morning.

By then Sarah had begun moving to the guest room on the worst snoring nights. She did not frame it as moving. She framed it as sleeping somewhere else for that one night. Within a year, the somewhere-else nights outnumbered the bedroom nights. The physician noted the snoring and recommended evaluation for sleep apnea. A covering urgent-care physician wrote the referral, two years later. The printed sheet sat on the kitchen counter for three days, then went into a stack of papers, then disappeared. The intention formed, persisted, and never became an appointment.

At fifty-three, the morning walk with the dog ended — the dog died, meetings moved earlier, winter came — and by spring, he was no longer the kind of person who walked in the morning. At fifty-four, the social calendar narrowed without anyone noticing.

In June of his fifty-sixth year, in the elevator at his office, he felt a pressure in his chest. It resolved when he reached his floor. He attributed it to the elevator. It happened again in August. The body had spoken. The hearing had not registered. By the time it had happened four times, he had stopped associating it with the

elevator and begun associating it, vaguely, with stress.

Sarah, who had been observing him for thirty years and could read his particular forms of avoidance, asked him in October whether he had followed up on the cardiologist appointment her own physician had suggested two years earlier. He said he would. He wrote *cardio* in his calendar. That Monday was three days before the pressure returned after dinner and did not pass — before he collapsed in the living room and Sarah called 911. Three days before the alarm rang in Room 7.

None of it felt like a crisis, or looked like one. Daniel went to work. He raised his children. He was present, functional, and, by any ordinary measure, fine.

He was also, in the language of physiology, quietly losing ground. One year at a time. One deferred appointment at a time. One unrenewed habit at a time. He did not know there was a margin.

This is what clinical medicine almost never names: not crisis, not catastrophe, but drift — the slow movement of a life in one direction, unnoticed, until the direction becomes the destination.

- 44 BP 128/84. Borderline. Follow-up not scheduled.
- 46 Quits tennis. Attrition, not decision.
- 48 BP 138/88. Medication, intermittent.
- 50 Weight up eighteen pounds. Snoring noted.
- 52 Sleep apnea referral. Never made. Coronary calcium zero, not repeated.
- 56 Room 7. ApoB 142, measured for the first time.

Each reading was documented. Each was filed. None was read as a sequence.

Room 7 • The Night of the Alarms

Ten years later, I was standing at his bedside just after two in the morning, watching the vasopressor drip and reading the numbers on the monitor above his head.

The procedure had worked. The stent was in. The blocked artery had been reopened within thirty-one minutes of his arrival, which is an extraordinary achievement — the product of a protocol refined over decades, a catheterization lab staffed and ready in the middle of the night, and tools that did not exist a generation ago.

And yet there I was. Because a successful procedure and a stable patient are not always the same thing.

His blood pressure was low but not critically so. His heart rate was elevated, working harder than it should need to after surviving a major attack. His oxygen saturation was acceptable — not the effortless 98 percent of a body in good health, but *acceptable*. The nurse was adjusting the medication that was doing the work his cardiovascular system could not fully do on its own.

He was awake. His eyes found me when I came to the bedside. That was a good sign.

I looked at him the way I look at every patient in those first moments: past the numbers, to the person. Trying to read the qualitative information that no monitor captures. How much effort is this costing him right now? What does he have left?

The honest answer, looking at Daniel, was: not much.

Not because of the artery. The artery had been fixed. Because of the ten years before the artery.

The decade's accumulations — the untreated sleep apnea, the abdominal weight, the slowly rising pressure, the fitness lost so quietly it had never announced its departure.

None of those things had caused the blockage on their own. Together, over ten years, they had determined what kind of body would be forced to survive it.

The Illusion of Sudden Illness

To the families of the patients I care for, serious illness almost always arrives as an ambush.

One day, someone is present and functional, moving through the ordinary routines of life. The next day, they are in an intensive care unit, surrounded by machines, and everything that seemed solid has revealed itself as precarious. The speed of it is part of what makes it terrifying. The apparent randomness of it is part of what makes it so difficult to make sense of afterward.

I understand why it feels that way. The event — the collapse, the chest pain, the call from the hospital at eleven at night — arrives without warning. The crisis is sudden.

But the process that produced the crisis rarely is.

In Daniel's case, the artery that closed had been narrowing for the better part of a decade. The process is called atherosclerosis, and it is among the most studied phenomena in all of medicine. Small lipid particles in the bloodstream penetrate the inner lining of arterial walls and become oxidized. The immune system responds. White blood cells infiltrate the wall. Inflammation follows. Over months and years, a plaque forms — a complex structure of lipids, immune cells, and scar tissue that slowly protrudes into the channel where blood flows.

For a long time, the artery compensates. It remodels itself around the obstruction, expanding outward to maintain flow. Patients feel nothing. Stress tests can be normal. Even sophisticated imaging can miss plaques that are growing but have not yet significantly narrowed the channel.

Then something changes in the plaque's structure. A thin fibrous cap overlying the lipid core becomes unstable — triggered by inflammation, elevated blood pressure, or the mechanical stress of a heart working too hard. The cap ruptures. The plaque's contents are exposed to circulating blood. A clot forms with terrifying speed. An artery that has been gradually narrowing for years becomes completely blocked.

Blood flow stops. Muscle in the heart begins to die.

And in a living room in a quiet suburb, a man who thought he was fine realizes he is not.

The event is dramatic. The process is gradual. And the gap between those two facts is where the most important medicine happens — or doesn't.

What Reserve Actually Is

Reserve is, at its most basic, the body's capacity to do more than the minimum. The heart's ability to increase its output when illness demands more circulation. The lungs' capacity to deepen their exchange when infection strains their function. The immune system's readiness to mount a fast and effective response when pathogens invade. The muscles' ability to maintain mobility and support recovery after a week spent confined to a bed.

The same logic applies to the brain. Decades of sustained cognitive engagement, vascular integrity, and restorative sleep build what neurologists call cognitive reserve — the capacity to maintain function in the presence of accumulating pathology. Two people with identical amyloid burdens can present entirely differently. The difference is not the disease. It is the reserve available to route around it.

Reserve is not binary. It is dynamic. It exists on a continuum and responds to how you live over decades.

It can be built. Aerobic exercise increases cardiovascular reserve in ways that are structural, measurable, and directly predictive of survival. Resistance training preserves muscle mass that would otherwise decline at a rate the body accepts but that the ICU reveals as consequential. Restorative sleep supports the hormonal, immune, and metabolic cycles that regulate nearly everything else. Strong human connection has measurable effects on inflammatory markers, cortisol patterns, and immune surveillance.

And it can be eroded. Chronic inactivity weakens the cardiovascular system progressively. Metabolic disease damages blood vessels, nerves, and organs with quiet efficiency. Poor sleep disrupts the hormonal cycles that govern metabolism and immune function. Chronic unmanaged stress maintains the body in a state of low-level physiological crisis that eventually extracts a cost from nearly every system it touches.

For years or even decades, these changes are invisible. People function. They go to work. They raise their children. They pursue their interests. They do not, most of the time, feel like people whose bodies are drifting toward anything.

The Industry That Has the Order Wrong

There is a booming culture around longevity. You have probably encountered it.

Cold plunges. Continuous glucose monitors worn by people without diabetes. Biological age tests. NAD⁺ infusions. Sleep tracking, HRV tracking, VO₂ max testing, fasting windows calibrated by algorithms. A vocabulary of optimization that has crossed from the medical literature into mainstream culture with extraordinary speed.

I am not dismissing all of it. Some of it is grounded in real biology. What I am questioning is the order.

The optimization culture starts at the top — selling the most sophisticated interventions to people who have not built the foundation those interventions depend on. It inverts the biology.

Not because they were careless, but because the culture is more enthusiastic about the upper tiers than the lower ones. The lower tiers are not new. They cannot be sold as breakthroughs. They require consistency across decades rather than a purchase and a protocol.

In the ICU, they are the difference between a patient who survives and one who does not.

The most important lesson the ICU has taught me is not what to do in a crisis. It is what to do in the thirty years before one.

What Happened to Daniel

Daniel spent nine days in the ICU. His recovery was longer and harder than it needed to be, not because the cardiac intervention had failed, but because his body had less capacity than it needed to recover well.

He got through it. He went home. He was, in the discharge paperwork, a success.

Sarah, who described his sleep to me at two in the morning while he lay in Room 7, had, for ten years, slept beside a man with untreated sleep apnea. The admission workup would eventually quantify the severity of that sleep apnea at thirty-seven events per hour. After discharge, she became the person beside him each morning at six-fifteen, on the same sidewalk, in the same neighborhood. She had asked him, she said, if they could take the route that passed the park. He had said yes. This was not a small thing. It was, I thought, the actual structure of his recovery — not the statin, not the CPAP, not the cardiac rehab, but the person who had called the ambulance and was now walking next to him every morning.

“I didn’t realize,” he said. “I thought I was basically fine.”

It is the most common thing a person says when they understand, for the first time, what the margin they had been living on actually was.

The ten years before Room 7 felt fine because they were not acutely terrible. His blood pressure was elevated, not dangerous. His extra weight was noticeable, not alarming. His sleep was poor, not catastrophic. Each data point, in isolation, was manageable.



Sarah found me in the cafeteria three weeks later. “Everything that happened to him,” she said, looking at the menu board over the counter, not at me, “was it in his chart?”

I told her yes. Most of it. The blood pressure. The weight gain. The sleep disruptions she had described to me in Room 7. The lipid panel that had existed, every year, for a decade.

“Then why,” she said, “did it take Room 7 for someone to look at the whole thing?”

I did not have a clean answer for her. I told her something that was true but not enough — that the chart reads “visits,” not “directions,” and that the tools to ask her question exist but are rarely used together.

She nodded. “Okay,” she said.

Someone had all those numbers. Why didn’t anyone tell us what they meant together?

Jack • Eight Years Before the Corridor

Jack was forty-nine years old and had never been admitted to a hospital when he sat down across from me in the outpatient clinic.

He was an architect, and not alarmed by the consultation. A corporate health screening had returned several flags on a basic metabolic panel. His resting heart rate, measured by the fitness watch he trusted implicitly, was fifty-four beats per minute. He walked regularly. He took the stairs. He was not wrong about any of this. He was wrong about what it meant.

His VO₂ max, measured that afternoon on a graded treadmill protocol, was 34.2 mL/kg/min — the bottom quartile for a forty-nine-year-old man.

“I don’t think of myself as out of shape,” he said.

“You aren’t,” I told him. “But the shape you can feel from the inside is not the same as the capacity that meets a crisis — and the gap between them stays invisible until the body is asked to survive something.”

“What does that mean, practically?” he asked.

That question — those four words, in that order, in a routine outpatient consultation at forty-nine — is what separates Jack’s story from Daniel’s. Not intelligence. Not discipline. Not access to better medicine. What Daniel never had was the conversation that reframed his situation from “basically fine” to “a direction that can be changed.”

I ordered his ApoB. It came back at 118 mg/dL — elevated relative to his LDL-C of 112, which had been reviewed at three consecutive annual physicals and found unremarkable. His sleep study showed no diagnosable apnea, but documented four to six respiratory disturbances per hour — enough to fragment slow-wave sleep and blunt the overnight hormonal cycles that govern metabolic recovery.

“So what do I do?” he asked.

I told him. Not everything. Not the whole framework at once. The base. Walk faster, and with intention. Lift weights against resistance twice a week. Sleep as if it matters. Measure ApoB and treat it to target. Come back in six months.

He came back in six months. Jack had not become an athlete. He had become someone whose week now sent his body repeatable signals: faster walks, two fixed lifting sessions, a sleep schedule protected instead of renegotiated every night. The next layer added only when the one beneath it had begun to hold.

Eight years later, Jack walked out of this building through a corridor that runs a hundred and thirty feet from the room where another patient spent eleven days fighting for his life. His VO₂ max was in the fifty-fourth percentile. His ApoB was 72. His body, which had been moving in one direction, was now quietly moving in another.

No alarm had sounded — which was exactly why Jack’s story mattered.

You are more likely to be Jack before the conversation than after it. Most people reading this have a sentence they use about their own body — *I’m just tired. I don’t recover like I used to. I’m fine, I just need to lose ten pounds* — offered as explanation, not complaint, too ordinary to mention. It is the most common signal in adult medicine, and the most underread.

NOW WHAT?

Build Reserve — Direction

→ **Clinical action.** At your next visit, bring the three most recent values for your blood pressure, fasting glucose, and lipid panel. Ask your physician to plot them across time, not against the single-visit threshold. The conversation this generates is different from the one a single reading produces.

→ **Behavioral minimum.** Name one thing you have been tolerating because it is not yet bad enough to act on. Write it down. That is the sentence Daniel could not write at forty-six.

→ **Carry forward.** The chart records where you are; it is silent on where you are going. That silence is the danger.



THE RESERVE SYSTEM

Find out where your own reserve is being spent.

Chapter 1 names the problem: drift. The Reserve System Score™ gives you a way to organize it — a short, educational assessment across capacity, load, and reserve, built from the same framework as the book.

[Get Your Reserve Score →](#)

Before the Alarm: What the ICU Reveals About Real Longevity, by M. K. Jarbou, MD, publishes September 7, 2026.

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